Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
7703021000, 7897319900
www.purepathlab.in Gmail: help@purepathlab.in
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

Mohd Smile

Age / Sex : 50 years / Male
Reg. no. : PPL-20261014-02643

PPL-20261014-02643
Registered on : 14/10/2026 05:21 PM
Collected on : -
Received on : -
Reported on : 14/10/2026 07:25 PM

HAEMATOLOGY
COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**L**	**12.7**	**g/dl**	**13 - 17**
TOTAL LEUKOCYTE COUNT		10500	cumm	4800 - 10800
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS		57	%	40 - 80
LYMPHOCYTE		35	%	20 - 40
EOSINOPHILS		02	%	1 - 6
MONOCYTES		06	%	2 - 10
BASOPHILS		00	%	< 2
PLATELET COUNT	**L**	**0.27**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT		5.41	million/cumm	4.5 - 5.5
HEMATOCRIT VALUE, HCT	**L**	**35.6**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV	**L**	**65.8**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH	**L**	**23.5**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**35.7**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		11.5	fL	6.5 - 12
P-LCR	**H**	**50.7**	**-**	**19.7 - 42.4**
R.D.W. - CV	**H**	**36.4**	**%**	**11.6 - 14**
P.D.W.	**H**	**50.7**	**fL**	**9.6 - 15.2**
R.D.W. - SD	**L**	**36.4**	**fL**	**39 - 46**

PURE PATH LAB
Palce of Accuracy & Your Satisfaction

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Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

CLINICAL PATHOLOGY

TEST	VALUE	UNIT	REFERENCE
URINE ROUTINE EXAMINATION			
QUANTITY	15	ml	-
COLOUR	Yellow	-	Pale Yellow
TRANSPARENCY	Clear	-	Clear
SPECIFIC GRAVITY	1.015	-	1.005 - 1.03
PH	5.0	-	5 - 7
LEUKOCYTES	Absent	-	Absent
BLOOD	Absent	-	Absent
PROTEIN / ALBUMIN	Absent	-	Absent
SUGAR / GLUCOSE	+++	-	Absent
KETONE BODIES	Absent	-	Absent
BILIRUBIN	Absent	-	Absent
NITRITE	Absent	-	Absent
R.B.C.	Absent	/HPF	Absent
PUS CELLS	Absent	/HPF	Absent
EPITHILIAL CELLS	Absent	/HPF	Absent
CASTS	Absent	-	Absent
BACTERIA	Absent	-	Absent
OTHERS	Absent	-	Absent

BIOCHEMISTRY
KIDNEY FUNCTION TEST (KFT)

TEST	VALUE	UNIT	REFERENCE
SERUM PHOSPHORUS	3.02	mg/dl	2.5 - 4.5
SERUM CREATININE	1.09	mg/dl	0.72 - 1.43
SERUM UREA	26.7	mg/dl	18 - 55
SERUM URIC ACID	6.43	mg/dl	3.5 - 7.2

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BIOCHEMISTRY
KIDNEY FUNCTION TEST (KFT)

TEST		VALUE	UNIT	REFERENCE
SERUM SODIUM		139.2	mmol/L	136 - 146
SERUM POTASSIUM		3.88	mmol/L	3.5 - 5.1
ICALCIUM		1.29	mmol/l	1.13 - 1.33
RANDOM BLOOD SUGAR	**H**	**168.2**	**mg/dl**	**70 - 140**
PH		7.236	-	-

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

BIOCHEMISTRY
BLOOD SUGAR FASTING & PP

TEST		VALUE	UNIT	REFERENCE
FASTING BLOOD SUGAR	**H**	**168.2**	**mg/dl**	**70 - 100**
BLOOD SUGAR PP	**H**	**296.6**	**mg/dl**	**< 180 mg/dl**

Clinical Notes

Elevated glucose levels (hyperglycemia) are most often encountered clinically in the setting of diabetes mellitus, but they may also occur with pancreatic neoplasms, hyperthyroidism, and adrenocortical dysfunction. Decreased glucose levels (hypoglycemia) may result from endogenous or exogenous insulin excess, prolonged starvation, or liver disease.

Fasting Glucose	2 hours PP Glucose	Diagnosis
<100	<140	Normal
100 to 125	140 to 199	Pre Diabetes
>126	>200	Diabetes

A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.

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BIOCHEMISTRY
LIVER FUNCTION TEST (LFT)

TEST		VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)		0.66	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)		0.24	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)		0.42	mg/dl	0.2 - 1
SGPT (ALT)	**H**	**50.74**	**U/l**	**13 - 40**
SGOT (AST)	**H**	**40.66**	**U/l**	**0 - 37**
SERUM PROTEIN		7.96	g/dl	6.4 - 8.3
SERUM ALBUMIN		4.89	g/dl	3.5 - 5.2
SERUM ALKALINE PHOSPHATASE		117.3	U/l	44 - 147
A/G RATIO		1.59	-	1.1 - 2.1
GLOBULIN		3.07	g/dl	1.8 - 3.6
SGOT/SGPT		0.8	-	-

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in

Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

BIOCHEMISTRY
LIPID PROFILE

TEST		VALUE	UNIT	REFERENCE
TOTAL CHOLESTEROL	**H**	**205.5**	**mg/dl**	**125 - 200**
TRIGLYCERIDES		114.9	mg/dl	25 - 200
HDL CHOLESTEROL		51.25	mg/dl	35 - 80
LDL CHOLESTEROL	**H**	**131.27**	**mg/dl**	**85 - 130**
VLDL CHOLESTEROL		22.98	mg/dl	5 - 40
LDL / HDL		2.56	-	1.5 - 3.5

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BIOCHEMISTRY
LIPID PROFILE

TEST	VALUE	UNIT	REFERENCE
TOTAL CHOLESTEROL / HDL	4.01	-	3.5 - 5
TG / HDL	2.24	-	-
NON-HDL CHOLESTEROL	154.25	-	-

Total Cholesterol - Interpretation

Desirable: <200 mg/dL [<5.2 mmol/L] Borderline: 200–239 mg/ dL [5.2–6.1 mmol/L] High risk: >240 mg/dL [>6.2 mmol/L]

Physiological basis

Cholesterol level is determined by lipid metabolism, which is in turn influenced by heredity, diet, and liver, kidney, thyroid, and other endocrine organ functions.

Interpretation

Increased in

Primary disorders: polygenic hypercholesterolemia, familial hypercholesterolemia (deficiency of LDL receptors), familial combined hyperlipidemia, familial dysbetalipoproteinemia. Secondary disorders: hypothyroidism, uncontrolled diabetes mellitus, nephrotic syndrome, biliary obstruction, anorexia nervosa, hepatocellular carcinoma, Cushing syndrome, acute intermittent porphyria. Drugs: corticosteroids.

Decreased in

Severe liver disease (acute hepatitis, cirrhosis, malignancy), hyperthyroidism, severe acute or chronic illness, malnutrition, malabsorption (eg, HIV), extensive burns, familial (Gaucher disease, Tangier disease), abetalipoproteinemia, intestinal lymphangiectasia.

Comments

Coronary heart disease (CHD) risk depends on the ratio of total cholesterol to HDL cholesterol. The ratio of LDL to HDL cholesterol has similar predictive ability. Treatment decisions should be based on absolute CHD risk. The risk reduction is proportional to the reduction in LDL cholesterol achieved with treatment.

Triglycerides - Interpretation

Physiological basis

Dietary fat is hydrolyzed in the small intestine, absorbed and resynthesized by mucosal cells, and secreted into lacteals as chylomicrons. Triglycerides in the chylomicrons are cleared from the blood by tissue lipoprotein lipase. Endogenous triglyceride production occurs in the liver. These triglycerides are transported in association with β-lipoproteins in very low-density lipoproteins (VLDL).

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Interpretation

Increased in

Hypothyroidism, diabetes mellitus, nephrotic syndrome, chronic alcoholism (fatty liver), biliary tract obstruction, stress, familial lipoprotein lipase deficiency, familial dysbetalipoproteinemia, familial combined hyperlipidemia, obesity, metabolic syndrome, viral hepatitis, cirrhosis, pancreatitis, chronic renal failure, gout, pregnancy, glycogen storage diseases types I, III, and VI, anorexia nervosa, dietary excess. Drugs: β-blockers, cholestyramine, corticosteroids, diazepam, diuretics, estrogens, oral contraceptives.

Decreased in

Tangier disease (α-lipoprotein deficiency), hypo- and abetalipoproteinemia, malnutrition, malabsorption, parenchymal liver disease, hyperthyroidism, intestinal lymphangiectasia. Drugs: ascorbic acid, clofibrate, nicotinic acid, gemfibrozil.

Comments

If the serum is clear, the serum triglyceride level is generally <350 mg/dL. Elevated triglycerides are now considered an independent risk factor for coronary artery disease and a major risk factor for acute pancreatitis, particularly when serum triglyceride levels are > 1000 mg/dL and can be seen when a primary lipid disorder that is exacerbated by alcohol or fat intake or by corticosteroid or estrogen therapy.

~~~ End of report ~~~

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